

Impact of Psychodermatological Disorders

Psychodermatology can be conceptualized as encompassing two interrelated domains:

1. **Primary psychiatric disorders** presenting with dermatological symptoms, such as in dermatitis artefacta (factitious disorder), skin picking disorder (excoriation disorder), or delusional infestation.
2. **Primary dermatological diseases** complicated by significant psychosocial comorbidities—such as anxiety, depression, or stress-related exacerbations—seen commonly in conditions like **psoriasis, atopic dermatitis, chronic urticaria, and prurigo**.

Psychological factors have been shown to influence both the onset and severity of these skin conditions. Stress, in particular, is associated with disease flare-ups and worsening symptoms. Under psychological stress, patients may develop maladaptive coping mechanisms—such as catastrophizing thoughts—that can exacerbate skin disease and increase the risk of suicidal ideation.

Moreover, comorbid depression and anxiety can lead to poor treatment adherence, resulting in suboptimal dermatological outcomes. The psychological burden extends beyond the individual, affecting family dynamics and overall quality of life.

Up to **98%** of patients with common skin diseases experience some psychological impact. Skin conditions ranked as the **fourth largest cause of nonfatal disease burden** globally in 2010, and the 2019 Global Burden of Disease study indicates that the burden—measured in years lived with disability (YLD)—has remained stable or increased for most skin disorders between 2010 and 2019.

Patients with dermatological conditions are at significantly higher risk for **depression, anxiety, suicidal ideation, stress, stigmatization, body**

dysmorphic concerns, and reduced health-related quality of life (HRQoL) compared to those without skin disease.

The term **psychoneurocutaneous medicine** has been introduced to reflect the complex interplay between the mind and skin, emphasizing the need for integrated care. Advancing this field promises improved diagnosis, treatment, and overall patient functionality and well-being.

Chronic skin diseases can profoundly affect **major life-changing decisions (MLCDs)**, including choices related to **childbearing, marriage, career, education, social engagement, physical activity**, and even **early retirement**.

Psychodermatological Consultations

Despite the high prevalence of psychological comorbidities, patients often face barriers in accessing appropriate psychodermatological care. Many clinicians lack training and resources to effectively identify and manage these complex cases.

The establishment of **multidisciplinary team (MDT) clinics**—integrating dermatology, psychiatry, psychology, and other allied health professionals—has demonstrated both **clinical effectiveness** and **cost-efficiency**. These clinics reduce misdiagnoses, eliminate redundant referrals, and prevent ineffective treatments, ultimately optimizing patient outcomes and resource utilization.